Business case

Customer value and commercial validation plan

Prepared 27 October 2026 · No prices, costs, sales volumes, or forecast are established.

What this document says

DrillLoop is a proposed local-first workspace. The initial segment hypothesis is hospital preparedness teams reviewing tabletop exercises. The likely user is an exercise evaluator or preparedness staff member; a preparedness lead or exercise sponsor is a candidate report approver. The budget owner and procurement path are unknown. Other emergency-management segments are possible future candidates, not validated customers.

What the product might add

Established platforms already offer after-action reports and corrective-action tracking. DrillLoop will test whether hospital evaluators can open the exact source record behind a proposed finding, see uncertainty, record a reviewer decision, and later check completion evidence. This is a hypothesis to validate, not a claim that those features are unique. A possible fit with Presight public-safety work needs internal validation; no integration or customer demand is established.

First commercial model to investigate: a per-exercise pilot fee. No price, delivery cost, staffing plan, customer volume, or willingness-to-pay evidence is established.

Commercial options to validate

Model	Current status	Questions to answer
Per-exercise pilot	First model to investigate; no price set	Who buys, what procurement route applies, and what one exercise requires.
Annual organization subscription	Later hypothesis; not priced	Exercise volume, reviewer roles, support, and renewal needs.
Private deployment	Later hypothesis; not priced	Hosting, identity, security, integration, and ongoing support requirements.
Custom templates or services	Later hypothesis; not priced	Configuration effort, domain expertise, facilitation, and training needs.

Inputs required before modeling economics

There are no confirmed prices, purchase volumes, delivery capacity, or operating costs, so this document does not present a revenue forecast. Gather these inputs with prospective users, buyers, and finance before preparing a scenario.

Input	Evidence to collect
Buyer and purchase	Identify report preparer, approver, budget owner, purchasing route, and actual willingness to pay.
Exercise volume	Use an organization-provided baseline; do not assume annual exercise counts.
Delivery effort	Record setup, template configuration, evaluator support, security review, and integration work.
Operating cost	Use finance-approved staffing, hosting, support, legal, and security inputs.
Customer value	Measure preparation time, corrections, citation quality, and action follow-up; distinguish capacity from cash savings.

Proposed validation plan

Question	Proposed evidence
Who owns the workflow and budget?	Interview prospective hospital-preparedness users; identify report preparer, approver, and budget owner.
Does traceability improve the current process?	Compare current practice and DrillLoop on the same permitted exercise records; measure preparation time, evidence lookup, and reviewer corrections.
How does it compare with an alternative?	Select one relevant alternative with a prospective partner and compare source-location precision, reviewer decisions, and closure evidence.
Does it generalize?	Use evaluator-labeled records from an unfamiliar exercise; report missed and incorrect findings, unsupported claims, and timeline agreement.
Can it be deployed safely?	Confirm data rules, identity, retention, access logging, hosting, and security requirements before using non-fictional records.

Known limits and risks

No customer interviews, baseline, buyer, partner, price, operating-cost model, or willingness-to-pay evidence is documented. Existing tools may already meet the need. The current rules were written for one fictional exercise and may not work on new scenarios. Evidence may be sensitive, and evaluators may not trust AI suggestions. A future study should measure source support, reviewer edits, action ownership, independent closure checks, and actual operating effort.

Research context

[1] FEMA, Homeland Security Exercise and Evaluation Program Doctrine, January 2020: https://www.fema.gov/sites/default/files/2020-04/Homeland-Security-Exercise-and-Evaluation-Program-Doctrine-2020-Revision-2-2-25.pdf

[2] FEMA Preparedness Toolkit, Exercise Evaluation: https://preptoolkit.fema.gov/web/training/evaluation

[3] California Department of Public Health, Power Loss scenario and hospital objectives: https://www.cdph.ca.gov/Programs/EPO/Pages/swmhe_scenarios_power_loss.aspx

[4] Veoci, After-action reporting product page: https://veoci.com/emergency-management/after-action-reporting/

[5] D4H, Emergency response team readiness software: https://www.d4h.com/team-readiness-software

[6] Mid-South Emergency Planning Coalition, 2023 Full Scale Exercise AAR/IP example: https://midsouthepc.org/wp-content/uploads/2024/03/Mid-South-HCC-2023-Non-Hospital-FSE-AAR.pdf